AC-04 Acute Coronary Syndrome / STEMI

MEDICAL-DIRECTOR APPROVED APP CONTENT | SOURCE-PDF REPLACEMENT | Reconciled October 16, 2026

Scope: Use for adults age 16 years or older with chest discomfort, an anginal equivalent, or an ECG concerning for acute coronary occlusion.

Rapid ACS and reperfusion pathway

1. Assess + monitor	ABCs, symptom-onset time, complete vital signs, SpO2, monitor, and pads when instability is present or anticipated.
2. 12-lead early	Obtain and transmit a diagnostic-quality 12-lead within 10 minutes when feasible. Repeat for persistent/recurrent symptoms, a nondiagnostic tracing, or deterioration.
3. Aspirin	Give 324 mg PO chewed unless true aspirin anaphylaxis, active major bleeding, or strong suspicion of acute aortic dissection. Complete a 324-mg loading dose if aspirin was taken before EMS arrival.
4. Nitroglycerin eligible?	SBP at least 110 mmHg and no shock, preload dependence/RV infarction, or prohibited recent PDE-5 inhibitor use: 0.4 mg SL every 5 minutes, maximum 3 doses.
5. Persistent ischemic pain	Paramedic: fentanyl 25-50 mcg IV/IO; repeat as needed to a maximum total of 100 mcg after aspirin and appropriate nitroglycerin. Use continuous monitoring.
6. Reperfusion destination	Early alert and transport to a catheterization-lab capable hospital or Claiborne Medical Center for hospital-directed thrombolytic treatment. Consider Covenant Health Air only when it reduces time to reperfusion-capable care.

Key treatment and handoff

Item	Approved action
Oxygen	Only for SpO2 below 90%, respiratory distress, shock, or another clinical indication.
Hypotension	Without pulmonary edema, give isotonic crystalloid in cautious 250-mL IV/IO aliquots with reassessment.

Target scene time is less than 15 minutes. Air activation must not delay aspirin, ECG acquisition, stabilization, or movement toward ground transport/rendezvous.

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SAFETY, LINKED PATHWAYS, AND DOCUMENTATION

Safety checks

- A normal or nondiagnostic initial ECG does not exclude acute myocardial infarction. Transmit early and repeat when the condition changes.

- Do not use new or presumed-new left bundle branch block alone as an automatic STEMI criterion.

- Withhold aspirin and nitroglycerin when acute aortic dissection is strongly suspected.

- Claiborne EMS does not independently administer thrombolytics, heparin, or a P2Y12 inhibitor under AC-04.

Linked pathways

Pathway	Use when
AC-05	Pulmonary edema/decompensated heart failure
AC-02	Bradycardia or poor perfusion
AC-03	Immediate pathway if pulseless

Documentation and release control

- Document the presenting condition, focused findings, rhythm/ECG when applicable, all interventions with dose/route/time/response, serial vital signs, reassessments, destination decision, and notifications.

- This source PDF replaces the legacy imported sheet and matches the approved Claiborne EMS app content reconciled October 16, 2026.

- Use the linked protocol whenever the patient crosses into another clinical pathway. Follow agency release governance, training requirements, and current medication/device availability.